

Session Note - March 25, 2026

Patient: Brian Okafor

Therapist: Dr. Susan Hartley, LPC

Presenting Concerns:

Patient presenting for intake evaluation following referral from PCP Dr. Adams for depression symptoms. Reports depressed mood for past 2 months following job loss. PHQ-9 score of 14 indicating moderate depression. Secondary concerns include insomnia and decreased appetite. This is the first episode of depression; no prior psychiatric history.

Session Summary:

Comprehensive intake evaluation completed. Gathered full psychosocial history, reviewed screening instruments, and established initial treatment plan. When completing Columbia Suicide Severity Rating Scale, patient answered NO to all screening questions including: wish to be dead, suicidal thoughts, suicidal thoughts with method, suicidal intent, and suicidal intent with plan. No history of self-harm or suicide attempts. No homicidal ideation. Patient is motivated to engage in therapy and expressed relief at taking this step.

Mood Assessment:

Current mood: 4/10 (low, discouraged)

Energy level: Low

Emotional themes: discouragement, loss of identity tied to career, worry about finances

Affect observed: Constricted, mildly dysphoric

Affect congruence: Congruent

Mental Status Examination:

Appearance: Well-groomed, appropriately dressed

Behavior: Cooperative, good eye contact

Speech: Normal rate and volume

Thought process: Linear, goal-directed

Thought content: Themes of loss and self-worth related to job loss

Perception: No abnormalities

Cognition: Intact, alert and oriented

Insight: Good - recognizes connection between job loss and mood

Judgment: Good

Risk Assessment:

Suicidal ideation: None. Columbia Suicide Severity Rating Scale completed - all items negative (wish to be dead: No, suicidal thoughts: No, suicidal thoughts with method: No, suicidal intent: No, suicidal intent with plan: No).

Self-harm: None. No history of self-harm or suicide attempts.

Homicidal ideation: None.

Risk level: Low.

Protective factors: supportive family, no substance use, motivated for treatment.

Interventions Used:

- Comprehensive intake assessment
- Psychoeducation about depression
- PHQ-9 and C-SSRS administration
- Initial treatment planning

Skills discussed: Sleep hygiene basics, behavioral activation concepts

Diagnoses:

Primary diagnosis: Major Depressive Disorder, single episode, moderate

Primary diagnosis code: F32.1

Diagnosis status: New diagnosis

Treatment Progress:

Treatment phase: Intake/Assessment

Recommended treatment: Weekly CBT sessions, 12-16 week course

Barriers: Financial concerns (lost insurance with job), transportation

Strengths: Motivated, supportive spouse, no substance use history

Plan:

- Start psychotherapy, weekly CBT sessions
- Next session: April 1, 2026
- Consider medication if no improvement in 4 weeks - coordinate with PCP Dr. Adams
- Sliding scale fee discussed and approved
- No immediate referrals needed
- Level of care: Outpatient